

Cancer Genetic (CGX Testing)

Cancer Genetic Testing

Consult Process

1. Verify patient's date of birth
2. Verify patient's full name
3. Start by asking a general question of, "Has anyone in your family, including yourself, ever had a cancer of any kind?" If they answer affirmatively, ask what type of cancer. "Are you still currently being treated for this cancer?"
4. If, when they answer they say no, or are not sure, begin to inquire a little more by asking if their mom or dad has had any cancer. You may be surprised at how much more information you obtain using this method.
5. Next, ask if their siblings have had any cancers, going through the list again.
6. Lastly, ask about grand-parents, children, nieces and nephews. In this manner, they will begin thinking of these people individually which can cause them to remember more than they may have with the more general questions.
7. Verify and confirm medications patient is currently taking.
8. Ask for any known drug allergies and previous medication side effects.
9. Does patient have personal history of surgical history?

Cancer Genetic Testing

How do we decide if the patient is approved for the test?

In addition to the patient, look for 1 or more family member(s) with the same cancer, or *within the same possible syndrome*. So, a total of 2 people *including* the patient.

All CGX consults are suggested to have, in this order:

- 1) A personal code: active treating diagnoses (C-Codes) , personal history (Z-Codes)
- 2) A family history code and,
- 3) A screening for genetic diseases code: an example is **Z13.79**.

CGX ICD10 Codes

The following are commonly used ICD 10 Codes. Please go to www.icd10data.com for a comprehensive list.

C22.0 - Liver Cell Carcinoma
C22.4 - Other sarcomas of Liver
C55 – Malignant neoplasm of uterus, part unspecified
C50.919 - Malignant neoplasm of unsp site of unsp female breast
C16.9 - Malignant neoplasm of stomach, unspecified
C25.9 Malignant neoplasm of pancreas, unspecified
C23 - Malignant neoplasm of gallbladder
C44.300 - Unspecified malignant neoplasm of skin of unspecified part of face
C43.9 - Malignant melanoma of skin, unspecified
C44.91 - Basal cell carcinoma of skin, unspecified
C71.9 – Malignant neoplasm of brain, unspecified
C61 - Malignant neoplasm of prostate
C56.1 – Malignant neoplasm of right ovary
C56.2 – Malignant neoplasm of left ovary
C67.3 – Malignant neoplasm of posterior wall of bladder
C67.4 – Malignant neoplasm of posterior wall of bladder
C81.70 – Other Hodgkin lymphoma, unspecified site
C18.9 – Malignant neoplasm of colon, unspecified
C34.11 – Malignant neoplasm of upper lobe, right bronchus or lung

Z85.820- Personal history of melanoma cancer
Z85.038- Personal history of colon cancer
Z85.07 – Personal history of Pancreatic cancer
Z85.00- Personal history of cancer of unspecified digestive organ
Z85.3- Personal history of breast cancer
Z85.850- Personal history of thyroid cancer
Z85.528- Personal history of kidney cancer
Z85.43- Personal history of ovarian cancer
Z85.42- Personal history of uterine cancer
Z85.44- Personal history of female genital organ
Z85.79 - Personal history of Lymphoma
Z85.51 - Personal history of bladder cancer

Z80.42- Family history of prostate cancer
Z80.3- Family history of breast cancer
Z80.6 - Family history of Leukemia
Z80.8- Family history of cancer of other organ systems (Ex: Thyroid) Z80.51- Family history of kidney cancer

Cancer Genetic Testing Syndromes

Descriptions

Hereditary Cancer Syndromes

Hereditary Breast & Ovarian Cancer Syndrome

The most common type of inherited [breast cancer](#) is hereditary breast and [ovarian cancer](#) syndrome (HBOC). HBOC is caused by mutations in the BRCA1 and BRCA2 genes. A woman with an inherited mutation in the BRCA can get breast or ovarian cancer and a man can have breast or prostate cancer. Sometimes pancreatic cancer and melanoma can be developed from this gene. <https://www.cancer.net/cancer-types/hereditary-breast-and-ovarian-cancer>

Cowden Syndrome

Cowden syndrome (CS) is characterized by multiple tumor-like growths and an increased risk of certain cancers. The majority of patients with CS develop small hamartomas, of the skin and mucous membranes, but these growths can also occur in the intestinal tract, brain, breast, uterus and thyroid. People with Cowden syndrome have an increased risk of developing certain types of cancer, *including melanoma and cancers of the breast, thyroid, endometrium (uterus), kidney, colon, and rectum*. Cowden syndrome is usually caused by mutations (changes) in the PTEN gene. Also called Cowden disease and multiple hamartoma syndrome. <https://www.cancer.gov/publications/dictionaries/cancer-terms/def/cowden-syndrome>

Lynch syndrome (hereditary Non-polyposis Colorectal Cancer Syndrome)

[Lynch syndrome](#) (HNPCC) is characterized by early age onset [colorectal cancer](#) and [uterine](#) (endometrial) cancer as well as other extracolonic tumors. There is also an increased risk of developing other types of cancers, such as *endometrial (uterine), gastric (stomach), ovarian, small bowel (small intestines), pancreatic, prostate, urinary tract, kidney, bile duct, and brain cancers*. [https://www.cancer.net/cancer-types/lynch-syndrome#:~:text=What%20is%20Lynch%20Syndrome%3F,polyposis%20colorectal%20cancer%20\(HNPCC\).](https://www.cancer.net/cancer-types/lynch-syndrome#:~:text=What%20is%20Lynch%20Syndrome%3F,polyposis%20colorectal%20cancer%20(HNPCC).)

Cancer Genetic Testing Syndromes

Descriptions...continued

Li-Fraumeni Syndrome

Li-Fraumeni Syndrome (LFS) is a rare genetic condition characterized by an increased risk of developing multiple types of cancer. The cancers that occur in LFS can be diagnosed during all stages of life. The most common types of cancer associated with LFS include:

- Soft tissue sarcomas
- Breast cancer
- Leukemia
- Lung cancer (Carcinoma/Sarcoma)
- Brain tumors
- Adrenal gland cancer

An increased risk for melanoma, Wilms' tumor, which is a type of kidney cancer, and cancers of the stomach, colon, pancreas, esophagus, lung (non-environmental), and gonadal germ cells (sex organs) have also been reported.

<https://www.cancer.net/cancer-types/li-fraumeni-syndrome#:~:text=LFS%20is%20a%20hereditary%20genetic,for%20a%20protein%20called%20p53>.

Von Hippel-Lindau disease

[Von Hippel-Lindau disease](#) (VHL) is a multi-system disorder characterized by abnormal growth of blood vessels (called hemangioblastomas or angiomas). Other types of tumors can develop in the adrenal gland, kidney and pancreas.

Multiple endocrine neoplasias

[Multiple endocrine neoplasia](#) (MEN) syndromes received their name because they predispose people to develop tumors of the endocrine glands.

There are two types of MEN syndromes: MEN1 (typically characterized by growths of the pituitary, parathyroid, and pancreas) and MEN2 (typically characterized by medullary [thyroid cancer](#)).

Familial adenomatous polyposis (FAP)

Familial adenomatous polyposis (FAP) is a colon cancer predisposition syndrome in which many precancerous colon polyps (called adenomas) are seen in the gastrointestinal tract. It can also be associated with osteomas and desmoids.

Cancer Genetic Testing Syndromes

Basal Cell Nevus Syndrome

Basal cell nevus syndrome refers to a group of irregularities caused by a rare genetic condition.

It affects the: Skin, endocrine system, nervous system, eyes and bones

Other names for basal cell nevus syndrome include:

Gorlin syndrome, Gorlin-Goltz syndrome and nevoid basal cell carcinoma syndrome (NBCCS)

Basal cell nevus syndrome is also responsible for the development of other cancers early in a person's life, including: **medulloblastoma** (a malignant brain tumor, usually in children) **breast cancer**, **non-Hodgkin's lymphoma (NHL)**, **ovarian cancer**

Peutz-Jeghers Syndrome

Peutz-Jeghers syndrome (PJS) is an inherited condition that puts people at an increased risk for developing hamartomatous polyps in the digestive tract, as well as cancers of the breast, colon and rectum, pancreas, stomach, testicles, ovaries, lung, cervix, and other types listed below.

Information provided by Dr. Rachael Ollivierre-Agard

Diabetes / Obesity Testing

Diabetes/Obesity Testing Criteria

1. Verify patients date of birth and full name.
2. Explain to the patient the reason for the call (they have an interest in obtaining Genetic testing for Diabetes and Obesity). This should have already been done before you speak with them but just in case be ready to explain.
3. Start by asking their Weight and Height so that you will be able to calculate the BMI of the patient. (There is an app called [Mediquations](#) that you can use for calculations. Feel free to use another one if you prefer).
4. Start by asking a general question of, “Has anyone in your family, including yourself, ever had or currently have diabetes type 2, prediabetes, HTN, hypercholesterolemia, sleep apnea, coronary artery disease, heart attack, open heart surgery or stents placed?”
5. If, when they answer no, or are not sure, begin to inquire a little more by asking if their mom or dad has had any diabetes/obesity related, along with sibling or family member on the same side. You may be surprised at how much more information you obtain using this method.
6. Lastly, ask about grand-parents, children, nieces and nephews. In this manner, they will begin thinking of these people individually which can cause them to remember more than they may have with the more general questions.
7. Verify and confirm medications patient is currently taking.
8. Ask for any known drug allergies and previous medication side effects.

Diabetes / Obesity Genetic Testing

Summary

The patient is qualified for the test if their BMI is above 25 and they, along with at least two members on the same side of their family, have **one** of the following: Diabetes Mellitus 2, Prediabetes, Hypercholesterolemia, HTN, Sleep Apnea, Coronary Artery Disease, Obesity.

OR

The patient is qualified for the test if their BMI is below 25 and they have had coronary artery disease evidenced by having had a heart attack, open heart surgery, or stents placed and one of the following: Diabetes Mellitus 2, Prediabetes, along with at least two members on the same side of their family, have **two** or more of the following: Diabetes Mellitus 2, Prediabetes, Hypercholesterolemia, HTN, Obesity, Sleep Apnea or Coronary Artery Disease.

Diabetes/Obesity Suggested ICD Codes:

Suggested BMI ICD 10 Codes:

Z68.25 - Body mass index [BMI] 25.0-25.9, adult

Z68.26 - Body mass index [BMI] 26.0-26.9, adult

Z68.27 - Body mass index [BMI] 27.0-27.9, adult

Z68.28 - Body mass index [BMI] 28.0-28.9, adult

Z68.29 - Body mass index [BMI] 29.0-29.9, adult

Z68.30 - Body mass index [BMI] 30.0-30.9, adult

Use Obesity ICD with BMI	
ICD 10	Description
E66.1	Morbid (severe) obesity due to excess calories
E66.9	Other obesity due to excess calories
E66.1	Drug-induced obesity
E66.8	Other obesity
E66.9	Obesity, unspecified

Diabetes/Obesity ICD 10 CODES

The following are commonly used ICD 10 Codes. Please go to www.icd10data.com for a comprehensive list.

E11.9 – Diabetes type 2 with no complications.

E78.00 – Hypercholesterolemia

Z82.49- Family history of coronary artery disease

Z86.79 – Personal history of coronary artery disease

R73.03 – Pre-Diabetes

G47.30 – Sleep Apnea, unspecified

I25.10 – Atherosclerotic Heart Disease of native coronary artery disease

I21.9 – Myocardial Infarction

Z82.49- Family history of ischemic heart disease and other diseases of the circulatory system

Z83.3 – Family history of diabetes mellitus

Z83.42 – Family history of familial hypercholesterolemia

*Two or more family must fall on the same side

Mother

Maternal Grandmother

Maternal Grandfather

Maternal Aunt

Maternal Cousin

Maternal Uncle

ETC

Father

Paternal Grandmother

Paternal Grandfather

Paternal Aunt

Paternal Uncle

Paternal Cousin

ETC

And/or

Patients' siblings and children

-Add BMI in the HPI note and/or PMHx.

-Specify what type of Diabetes

-Mother and father do not qualify as family as they are not same side

Pharmacogenetic (PGx Testing)

PGX Testing

Click [Here](#) for Audio Training

What makes PGX Testing a medical necessity?

The test gives the provider a better understanding of how patients will respond to a medication which can be life saving. It takes away the guess work and the trial and error for the class of medications which are tested. It also cuts cost due to hospitalizations due to drug reactions and also decreases costs due to multiple medications that may not even work for that patient.

Who qualifies for PGX Testing?

Patients taking multiple drugs (3 or more) for 1 or multiple classes of medications and may not be experiencing the expected results. Also, if the patient has a history of adverse reactions to medications or a family history of such.

The types of medications tested include but are not limited to the following: Psychiatric, Analgesics, Cancer medications, cardiovascular medications (including anti-coagulants). Eg: Warfarin, Tricyclic antidepressants, codeine, Tamoxifen, Atomoxetine, Clopidogrel, Isoniazid, Irinotecan, Azathioprine, Gefitinib, HER-2 over expression in breast cancer and KRAS gene in colon cancer.

*It is highly suggested to provide ICD 10 codes for EACH CONDITION the patient is taking the medication for.

Example: Losartan - High Blood Pressure - Diagnosis Code: I10

Pulmonary Genetic Testing

Guidelines for Pulmonary Genetic Testing

1. Introduce yourself, verify the patients date of birth and full name.
2. Explain to the patient the reason for the call. They have an interest in finding out if there are genetic causes for Lung Disease.
3. Start by asking a general question of “Has anyone in your family including yourself ever had/have any lung disease leading to breathing problems like asthma, COPD, Interstitial lung disease, emphysema, Pulmonary Fibrosis, Ciliary Dyskinesia, Bronchiectasis, Cystic Fibrosis and Surfactant Metabolism Dysfunction.”
4. They may start answering and say no and you can reiterate by saying something like “so no one in your family has any breathing issues like asthma or any other disease. Your brothers, sisters, mom, dad, grandparents, aunts, uncles or children.”
5. Please also gather details about the patient’s symptoms pertaining to Lung issues such as: Cough, Dyspnea, Orthopnea, Shortness of Breath, Stridor, Wheezing, Periodic Breathing, Hyperventilation, Tachypnea, Asphyxia, Pleurisy, Cyanosis, Abnormal blood-gas level, Abnormal results of pulmonary function studies, Hypoxemia.
6. Also please confirm the patient information in the chart, for example: Medications, PSHx, Allergies, and Soc Hx.
7. Please collect the following information from each patient and place in chart: PMHx, FHx, ROS, and Vitals.

How do we decide if the patient is approved?

The patient and 2 family members on the same side must have related lung issues or in a related syndrome (A list of diseases and syndromes will be attached for your perusal).

[Click here for a list of diseases and syndromes.](#)

[Click here for list of Pulmonary ICD 10 Codes.](#)

Neurocognitive Genetic Testing

How do we decide if the patient is approved?

1. Patient must have a personal history of one of the diagnoses from Section 1 AND one family history from Section 1, but the code to be used for family history is (Z82.0) from section 3.

OR

2. Patient must have a personal history of one of the diagnoses from Section 2 AND one family history from Section 1, but the code to be used for family history is (Z82.0) from section 3.

(Please scroll down to see the sections)

[Click Here for Neurocognitive video training](#)

[Click Here for Letter of Medical Necessity sample](#)

Section 1:

A81.83	Fatal familial insomnia
F01.50	Vascular dementia without behavioral disturbance
F01.51	Vascular dementia with behavioral disturbance
F02.80	Dementia in other diseases classified elsewhere without behavioral disturbance
F02.81	Dementia in other diseases classified elsewhere with behavioral disturbance
F03.90	Unspecified dementia without behavioral disturbance
F03.91	Unspecified dementia with behavioral disturbance
G11.3	Cerebellar ataxia with defective DNA repair
G11.8	Other hereditary ataxias
G11.9	Hereditary ataxia unspecified
G12.21	Amyotrophic lateral sclerosis
G20	Parkinson's disease
G21.3	Postencephalitic parkinsonism
G21.4	Vascular parkinsonism
G21.8	Other secondary parkinsonism
G21.9	Secondary parkinsonism, unspecified
G24.9	Dystonia unspecified
G30.0	Alzheimer's disease with early onset
G30.1	Alzheimer's disease with late onset
G30.8	Other Alzheimer's disease
G30.9	Alzheimer's disease, unspecified
G31.09	Other Frontotemporal dementia
G31.83	Dementia with Lewy bodies
G32.81	Cerebellar ataxia in diseases classified elsewhere

Section 2:

F22	Delusional disorders
F32.9	Major depressive disorder single episode unspecified
F39	Unspecified mood [affective] disorder
F40.298	Other specified phobia
F41.9	Anxiety disorder unspecified
F68.8	Other specified disorders of adult personality and behavior
G47.0	Insomnia
G47.00	Insomnia unspecified
G47.01	Insomnia due to medical condition
G47.09	Other insomnia
G47.9	Sleep disorder unspecified
H53.72	Impaired contrast sensitivity
M04.8	Other autoinflammatory syndromes
M04.9	Autoinflammatory syndrome unspecified
M62.40	Contracture of muscle unspecified site
R13.10	Dysphagia unspecified
R25.1	Tremor unspecified
R25.8	Other abnormal involuntary movements
R25.9	Unspecified abnormal involuntary movements
R26.2	Difficulty in walking not elsewhere classified
R26.81	Unsteadiness on feet
R26.89	Other abnormalities of gait and mobility
R27.8	Other lack of coordination
R27.8	Other lack of coordination
R29.6	Repeated falls
R32	Unspecified urinary incontinence
R41.0	Disorientation unspecified
R41.3	Other amnesia
R41.82	Altered mental status unspecified
R41.9	Unspecified symptoms and signs involving cognitive functions and awareness
R42	Dizziness and giddiness
R43.9	Unspecified disturbances of smell and taste
R44.3	Hallucinations unspecified
R45.0	Nervousness
R45.1	Restlessness and agitation
R45.2	Unhappiness
R45.3	Demoralization and apathy
R45.4	Irritability and anger
R45.6	Violent behavior
R45.89	Other symptoms and signs involving emotional state
R46.4	Slowness and poor responsiveness

R46.89	Other symptoms and signs involving appearance and behavior
R47.01	Aphasia
R47.89	Other speech disturbances
R53.83	Other fatigue
R63.0	Anorexia
R63.4	Abnormal weight loss
Z73.89	Other problems related to life management difficulty

Section 3:

(Must always include *plus* family hx disease from Section 1)

Z82.0	Family history of epilepsy and other diseases of the nervous system
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Neurocognitive Evaluation Panel™

Comprehensive Neurocognitive Genetics Panel

Detect 64 risk factor genes associated with neurocognitive disorders

An accurate, timely diagnosis of a nonreversible decline in mental function is crucial to ensure a personalized and data-driven treatment plan is provided to the patient. Early-stage support can help avoid unnecessary struggles and expenses associated with missed treatment opportunities.

Diax Labs offers our proprietary comprehensive neurocognitive evaluation panel, which examines 64 genes associated with an increased risk of developing neurocognitive disorders and detects both the diagnostic and risk factor genes for dementia, Alzheimer's disease, and Parkinson's disease. While no single test exists to diagnose dementia, DAP testing is an important component in the diagnostic process. Additionally, DAP testing rules out medical conditions that mimic the memory loss symptoms associated with dementia.

Parkinson's Disease

As a result of faulty genes being passed to a child by their parents, Parkinson's disease can run in families. Approximately 15% of people with Parkinson's have a family history of this disorder. Familial cases of Parkinson disease can be caused by mutations in the *LRRK2*, *PARK7*, *PINK1*, *PRKN*, *MAPT*, *GBA* or *SNCA* gene.

Alzheimer's Disease

One type of early-onset Alzheimer's disease is known as autosomal dominant Alzheimer's disease, or early-onset familial Alzheimer's disease (FAD), which affects approximately 3-5% of all people with Alzheimer's. What makes this type of early-onset Alzheimer's disease so unusual is that it is caused by a hereditary genetic mutation to one of three genes: *PSEN1*, *PSEN2* or *APP*. Genes that increase the risk of Alzheimer's disease are *APOE*, *TREM2* and *NOTCH3*.

Lewy-Body Dementia

Lewy body dementia, also known as dementia with Lewy bodies, is the second most common type of progressive dementia after Alzheimer's disease. Protein deposits, called Lewy bodies, develop in nerve cells in the brain regions involved in thinking, memory and movement (motor control). While much research is underway, it is widely accepted that familial cases of Lewy-Body Dementia can be caused by mutations in the *SNCA*, *APOE E4*, or *GBA* gene.

Frontotemporal Dementia

Frontotemporal Dementia (FTD) is the most common form of dementia for people under age 60. It represents a group of brain disorders caused by degeneration of the frontal and/or temporal lobes of the brain. FTD's estimated prevalence in the United States is around 60,000 cases, and many in the medical community remain unfamiliar with it. FTD is frequently misdiagnosed as Alzheimer's, depression, Parkinson's disease or a psychiatric condition. On average, it currently takes 3.6 years to get an accurate diagnosis. Up to 40% of patients have a positive family history with a diagnosis of dementia in at least one extra family member. Three major causal genes have been identified: *MAPT*, *GRN* and *C9ORF72*.

ALS

Approximately 5-10% of cases are due to genetic mutations and are inherited from a family member. If there are two or more family members with ALS, the disease is considered familial. There are an increasing number of gene mutations that have been identified both in familial and seemingly sporadic patients. The most commonly known ALS genes have been identified as: *C9orf72*, *FUS*, *TARDBP*, and *SOD1*.

Conditions

- Dementia
- Vascular dementia
- Cerebellar ataxia
- ALS
- Parkinson's
- Alzheimer's
- Frontotemporal lobe dementia
- Lewy Body dementia

Symptoms along with Family History:

- Depression
- Anxiety
- Fatigue
- Insomnia
- Sleep disorder
- Tremor
- Gait disturbance
- Repeated falls
- Amnesia
- Hallucinations
- Speech disturbance



844-867-8134 x diaxlabs.com

Immunodeficiency Genetic Testing

Guidelines for Immunodeficiency Genetic Testing

1. Introduce yourself, verify the patients date of birth and full name.
2. Explain to the patient the reason for the call. They have an interest in finding out if there are genetic causes for Immunodeficiency disorders.
3. Start by asking a general question of “Has anyone in your family including *yourself* ever had or currently have any immunodeficiency disorders like blood disorders-anemia, lupus, rheumatoid arthritis, frequent digestive problems, severe eczema, delayed growth and development?”
4. They may start answering and say no and you can reiterate by saying something like “So no one in your family has any immunodeficiency disorders such as sickle cell anemia, bone marrow failure or complications, rheumatoid or psoriasis arthritis, Di George’s syndrome, recurrent pneumonia or infections? Your brothers, sisters, mom, dad, grandparents, aunts, uncles or children.”
5. Also please confirm the patient information in the chart, for example: PSHx, FHx, Medications, Allergies, Vitals, Soc Hx, ROS.

How do we decide if the patient is approved?

1. The patient has an immunodeficiency disorder *and* (2) Family members with known primary immunodeficiency disorder.

OR

2. The patient has the below listed conditions of immunodeficiency symptoms *and* (2) Family members with known primary immunodeficiency disorder.

- Frequent and recurrent pneumonia, bronchitis, sinus infections, ear infections, meningitis or skin infections
- Inflammation and infection of internal organs
- Blood disorders, such as low platelet counts or chronic/long standing Anemia
- Digestive problems, such as cramping, loss of appetite, nausea and diarrhea
- Delayed growth and development
- Autoimmune disorders, such as lupus, rheumatoid arthritis or type 1 diabetes

- Multiple, unusual, or severe allergies
- Eczema
- Ectodermal dysplasia
- Abnormal hair
- Early- or very-early-onset inflammatory bowel disease or enteropathy
- Early-onset or severe autoimmunity
- Early-onset endocrinopathies
- Lymphoproliferative disease
- Hemophagocytic lymphohistiocytosis (HLH)
- Neurologic abnormalities
- Skeletal dysplasia
- Asthma

[Click Here for a list of Immunodeficiency ICD 10 Codes](#)

[Click Here for Immunodeficiency training video](#)

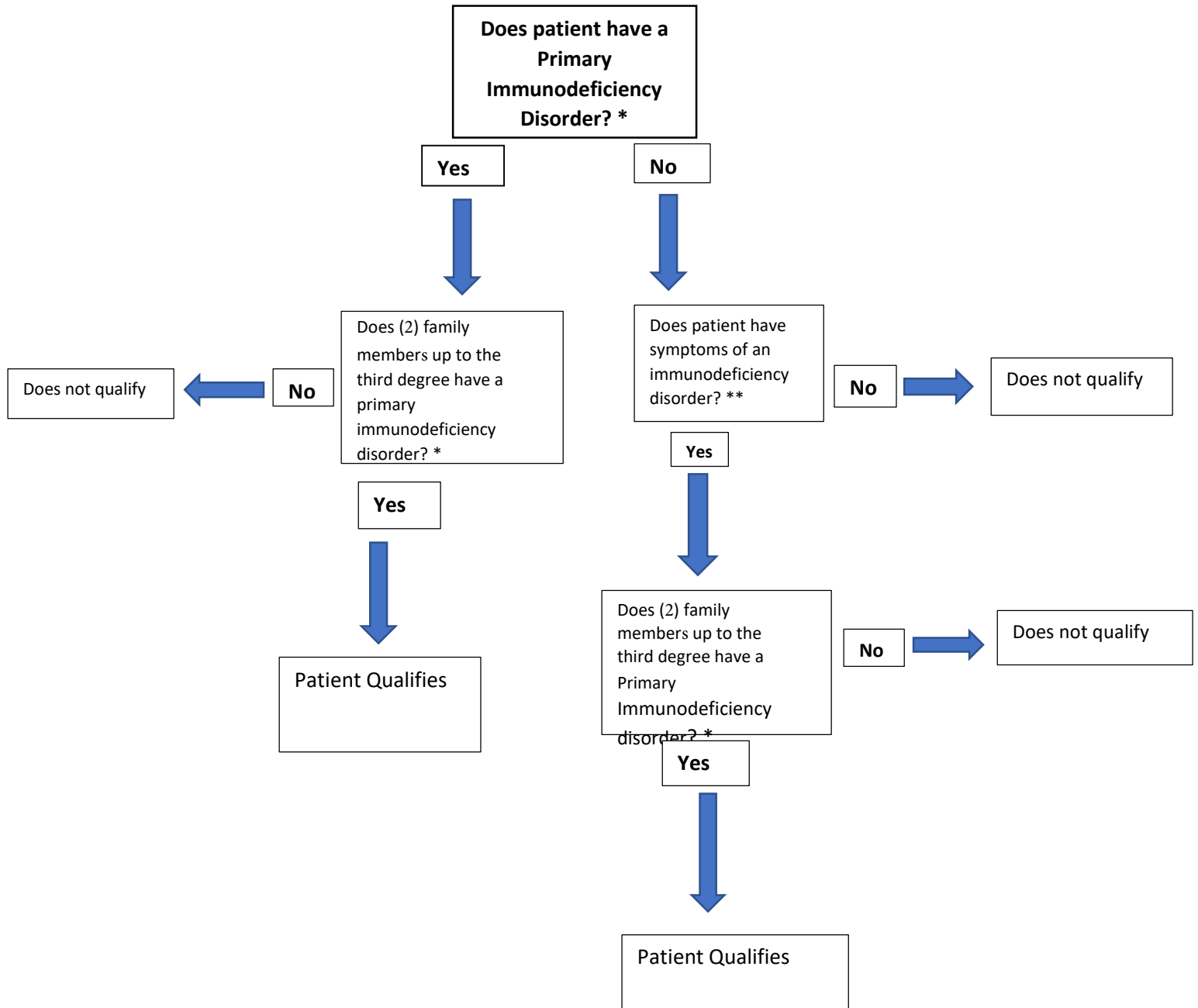
[Click Here for Letter of Medical Necessity sample](#)

[Click Here for a list of Immunodeficiencies](#)

List of Common Primary Immunodeficiency (Auto-Immune Disorders)

- 1. Type 1 diabetes**
- 2. Rheumatoid arthritis (RA)**
- 3. Psoriasis/psoriatic arthritis**
- 4. Multiple sclerosis**
- 5. Systemic lupus erythematosus (SLE)**
- 6. Inflammatory bowel disease**
- 7. Addison's disease**
- 8. Graves' disease**
- 9. Sjögren's syndrome**
- 10. Hashimoto's thyroiditis**
- 11. Myasthenia gravis**
- 12. Autoimmune Vasculitis**
- 13. Pernicious anemia**
- 14. Celiac disease**
- 15. Hemophilia**
- 16. Hemolytic anemia**
- 17. Sickle Cell anemia**
- 18. Lupus**
- 19. Chronic or long standing Eczema**
- 20. Recurrent Pneumonia/Bronchitis/Sinus Infections**
- 21. Severe/Chronic Asthma**
- 22. Autoimmune Cancers: Lymphoma, Leukemia, Myeloma**

Immunodeficiency Flow Chart



*Please remember these are guidelines of criteria and may not list all acceptable autoimmune/immunodeficiency conditions.

ICD Codes

ICD10 Codes

4 mins, 27 secs

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Summary:

(0:41) Make sure the codes you are inputting are billable codes.

A nonbillable code cannot be used as the first code. If you use a nonbillable code, it cannot be the first code in the list and it must be accompanied by a billable code.

(2:17) Be careful not to input the category or group code, such as E78.0, which is not a billable code. That is the category code, the actual billable code is E78.00.

(3:00) Ensure that if you are doing a consult for cardio, that you only use codes which apply to cardio. Or, CGX codes for CGX tests, etc.

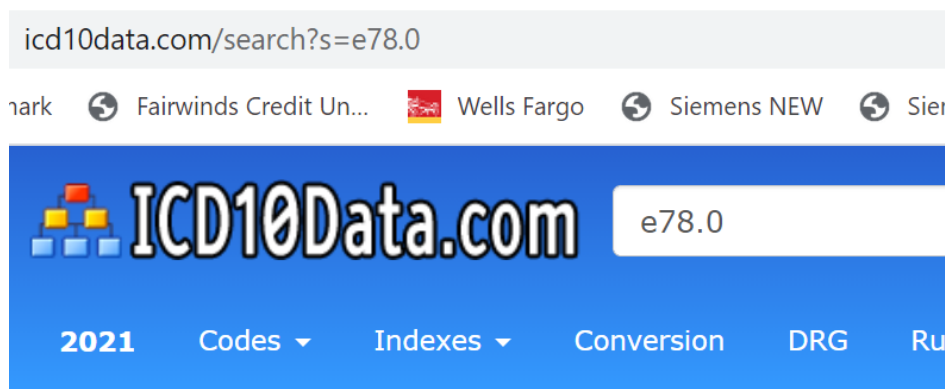
(7:08) PGX Codes. You must have one of the Z codes listed first, followed by an I code or an E code that are at the top of the list.

ICD10 Codes

ICD10Data.com is a free reference website designed for the fast lookup of all current American ICD-10-CM (diagnosis) and ICD-10-PCS (procedure) medical billing codes.

The 2021 ICD-10-CM/PCS code sets are now fully loaded on ICD10Data.com. 2021 codes became effective on **October 1, 2020**, therefore all claims with a date of service on or after this date should use 2021 codes.

<https://www.icd10data.com/>



3 result found:

Non-billable (red arrow) → **ICD-10-CM Diagnosis Code E78.0** 
Pure hypercholesterolemia

Billable (green arrow) → **ICD-10-CM Diagnosis Code E78.00**  [convert to ICD-9-CM]
Pure hypercholesterolemia, unspecified

Billable (green arrow) → **ICD-10-CM Diagnosis Code E78.01**  [convert to ICD-9-CM]
Familial hypercholesterolemia

Denial Codes

Denials: Include Codes and Notes

4 mins, 27 secs

Click [Here](#) to listen to Audio Training

Summary:

- Must include ICD 10 codes on Denials
- Cannot leave the codes blank
- Whatever conditions they may have, we must include those codes even if we are denying the test
- Must write notes. Cannot just say, “patient did not qualify.” Must provide a note as to why they did not qualify.
- Need to elaborate more on the conditions they do have.
- If the patient or their family does not have any history of disease pertaining to the test in question, the Z13.79 code can be used but this only if there is absolutely no personal or family history.